

DISCHARGE SUMMARY

PATIENT DEMOGRAPHICS

Name: Patient 46

Age: 49 years old

Sex: Female

MRN: 000-TEST-46

Admission Date: [Admission Date]

Discharge Date: [Discharge Date]

Length of Stay: 2 days

PRIMARY DIAGNOSIS

Acute appendicitis s/p laparoscopic appendectomy

SECONDARY DIAGNOSES

- Hypothyroidism, well-controlled
- Generalized anxiety disorder
- Mild post-operative pain

HOSPITAL COURSE

Patient 46 presented to Metro Medical Center via ED with RLQ pain, fever 38.2°C, and elevated WBC (13,200). Imaging studies (CT abdomen/pelvis) confirmed acute appendicitis without evidence of perforation. Patient was taken to OR on hospital day 1 for definitive management. Laparoscopic appendectomy performed without intraoperative complications; procedure remained laparoscopic (no conversion to open required). Three trocar sites placed: umbilical 10mm, RLQ 5mm, suprapubic 5mm. Operative time 28 minutes. EBL minimal. Post-op vitals stable. Patient tolerated clear liquid diet on POD#1 and was transitioned to regular diet on POD#2 without nausea/emesis. Pain controlled with oral analgesics. Trocar sites cleaned and dressed with sterile gauze; no signs of infection, erythema, or drainage noted at discharge. Patient ambulating independently, tolerating PO well, void pattern normal.

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Acetaminophen	500 mg	Q6H PRN pain/fever	10 days
Ibuprofen	400 mg	Q8H PRN (w/ food)	7 days
Levothyroxine	75 mcg	Q.d. (morning, empty stomach)	Ongoing
Sertraline	50 mg	Q.d. (morning)	Ongoing
Oxycodone	5 mg	Q4H PRN severe pain	3 days only

WOUND CARE & ACTIVITY RESTRICTIONS

- Keep all three trocar sites clean and dry; gentle soap and water cleansing permitted
- Leave adhesive strips (steri-strips) in place; will fall off naturally in ~5–7 days
- Notify office if any site shows purulent drainage, increasing erythema, or warmth
- NO lifting, pushing, or pulling >5 lbs × 2 weeks post-op
- NO strenuous exercise, running, or heavy lifting × 4 weeks
- Light walking encouraged for circulation
- Resume normal activities gradually; avoid contact sports × 6 weeks

DIET

- Started on clear liquids; advanced to regular diet as tolerated on POD#2
- No dietary restrictions at discharge; avoid foods that trigger reflux or discomfort
- Increase fiber and fluids to prevent post-operative constipation
- Avoid carbonated beverages × 1 week if bloating persists

FOLLOW-UP APPOINTMENTS

- Surgeon (General Surgery Clinic): in 2 weeks for wound check and post-op assessment
- Primary Care Physician: in 4 weeks for routine follow-up and medication reconciliation
- Gastroenterology: optional 6-week f/u if patient develops persistent GI symptoms (not routine)

RED FLAG SYMPTOMS — SEEK EMERGENCY CARE IF:

- Fever >101.5°F (39°C) or chills
- Spreading erythema, purulent drainage, or dehiscence at trocar sites
- Severe abdominal pain not relieved by prescribed analgesics
- Persistent nausea